

ROCHESTER
REGIONAL HEALTH

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October 20, 2020

RE:
Mark D Robinson
1268 Westside Dr
Rochester NY 14624-3828

Patient: **Mark D Robinson**
Date of Birth: **2/9/1976**
Date of Visit: **10/20/2020**

To Whom it May Concern:

Mark Robinson was seen in my clinic on 10/20/2020. He may return to work on 10/21/20 without restrictions.

If you have any questions or concerns, please don't hesitate to call.

Sincerely,

A handwritten signature in black ink, appearing to read "Michael C. Stanton" followed by "M.D." in a smaller, less distinct script.

Michael C. Stanton, M.D.

PROGRESS REPORT

Date: 10/20/2020

Patient: Mark Robinson
Diagnosis: Left distal biceps repair
Referring Physician: Michael Stanton, MD

DOB: 2/9/76
DOS: 7/1/2020
PCP:

SUBJECTIVE REPORT:

HPI: Reports injury while lifting at work. Surgery was performed soon after the injury. Currently he reports paresthesias of the forearm. Describes a mild discomfort of his distal forearm that is relatively constant, with intermittent sharp transient pain with use. **PN 10/20: No recent pain to report. Self-reports function to be 75%.**

Functional Limitations: Twisting of the forearm, turning a steering wheel, opening a door knob, has lifted and carrying minimally, feels it taking a plate from the microwave. **PN 10/20: No limitations with ADL activity and has begun to attempt lifting.**

Treatment to Date: Ibuprofen QAM

Activities/ Goals: Work: HVAC sales; golfing, motor cycle, yard work,

OBJECTIVE MEASUREMENTS:

Inspection: Incision well approximated and healed without scar hypertrophy

Palpation: General TTP to the incisions site

Joint Mobility: WNL

Flexibility: deferred

Functional Tests: Grip (L/R): Pos1- 80/77, Pos2- 120/130, Pos3- 127/129

Neurologic: Light touch intact and equal bilaterally; mild hyperesthesia noted

Self-Assessment Scale Score: DASH: 22% (improved from 63%)

	L		R	
	L	R	L	R
Elbow FLX	/	WNL	/	WNL
Elbow EXT	/	WNL	/	WNL

*denotes pain during motion; **denotes pain at end range

	L		R	
	L	R	L	R
Elbow FLX		4		4
Elbow EXT	4+	4+		4+

ASSESSMENT: Mark has demonstrated excellent compliance with attendance to PT and reports regular HEP performance. ADL function has normalized which is consistent with strength gains to this point of rehab. He will benefit from continued PT for progression of functional strength to restore full pre-morbid levels of function.

TREATMENT PLAN:

Skilled PT services to include but not limited to the following: patient education, HEP, therapeutic exercise, manual therapy techniques, functional movement, postural education, balance/proprioceptive activities, neuro-muscular re-education, and modalities PRN.

Frequency/Duration of treatment: Patient to continue Physical Therapy 1-2x/week for 4 weeks.

Thank you for allowing me the opportunity to treat your patient.

Jeff McGivern PT, MSPT

Date: 10/20/2020

I certify that the physical therapy services prescribed are medically necessary. I agree with the established POC noted above.
Please sign and return to Lattimore Physical Therapy and Sports Rehab at your earliest convenience.

Physician's signature: _____ Date: _____